Department of Health and Human Services

Centers for Medicare & Medicaid Services RFI: Health Technology Ecosystem

Vendor Comments

June 16, 2025

HHAeXchange

Introduction

For more than 17 years, HHAeXchange (HHAX) has been at the forefront of delivering better homecare ecosystem solutions by helping payers and providers achieve operational efficiency, increase compliance, and improve patient outcomes. In October 2024, HHAeXchange acquired Sandata Technologies (Sandata), who, for 45 years, has provided technology solutions for providers and payers in the homecare ecosystem, with the first Electronic Visit Verification (EVV) and aggregator deployment in 2010. Since then, the combined HHAeXchange/Sandata organization has successfully implemented a total of 31 state EVV programs. Of those implemented clients, we have supported 19 in achieving CMS certification, with five additional states awaiting their final certification letter from CMS (the other programs are not pursuing certification at this time). As a unified organization, we have the most state contracts and experience—as well as CMS-certified programs—in the market, and we are the premiere homecare management software provider for the Medicaid LTSS, FFS, and Customer-Directed populations.

Serving the Whole Ecosystem

We are the only vendor serving every level of homecare in the market–states, payer and MCO, provider agency, caregivers, members/customers, self-direction, and FMS.

HHAX is an organization completely dedicated to the homecare and Medicaid LTSS population. HHAX is unique in that we serve all stakeholders in the HCBS community, including Medicaid agencies, MCOs, FMS/FI/beneficiary-directed providers, personal care agencies, and certified home health agencies. Our focus is entirely on software solutions for these stakeholders to minimize the burden on beneficiaries, providers, and workers, while staying in compliance with CMS regulations and adapting to legislative changes and technological advances.

Our comments in response to the issued Request for Information (RFI) regarding the Health Technology Ecosystem are shaped by this specific lens of experience. We are uniquely situated to understand the various stakeholder burdens across the ecosystem, with an inherent understanding of the current Electronic Visit Verification (EVV) framework and related technology solutions that are already deployed and how the existing framework can be utilized to enhance current service delivery and quality; integrate fraud, waste, and abuse (FWA) initiatives; improve outcomes with the collection and analysis of social determinants of health (SDOH), care gaps, and other person-centered information; and leveraged for additional future initiatives as necessary, without the burden and expense of implementing a new framework.

While our scope of experience is more general than many of the highly specific and/or technical questions in the RFI, our heavy involvement with the impacted stakeholders allows us to have a “30,000 foot view” of the homecare ecosystem. Based on our work with all stakeholders, including provider agencies and their caregivers, we specifically wanted to provide commentary on C.3.b as we are privy to the challenges for compliance, privacy, access, and convenience these stakeholders face, along with the members they serve. We appreciate the Department’s foresight in soliciting feedback, and are grateful for the opportunity to submit comments.

Table of Contents

Introduction i

C. Providers, 3. Digital Identity 1

Existing Electronic Visit Verification Framework 1

Patient Privacy 2

Convenience and Access 2

Improve Outcomes 2

C. Providers, 3. Digital Identity

> *b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?*

Existing Electronic Visit Verification Framework

Where secure tools exist in the current ecosystem, we believe the best strategy would be to extend and maximize their effectiveness, rather than attempt to reinvent the wheel.

For example, within the homecare ecosystem, the 21st Century Cures Act mandated Electronic Visit Verification (EVV) for all Medicaid personal care and home health care services that require in-home visits. The software platform and tools supporting EVV are capable of securely and conveniently delivering data that improves health outcomes and quality while also reducing fraud, waste, and abuse, and CMS actions can help to achieve those advantages.

Specifically, the existing EVV framework can be utilized to enhance patient outcomes by more consistently collecting data on potential care risks and social determinants of health. Caregivers in the patient's home can provide these observations easily if they are part of a workflow and digital application they are already accustomed to using, enabling providers to analyze information in near-real time and take action promptly if appropriate. This proactive approach has the potential to greatly enhance quality of care, and coincides with CMS regulation 2442-F by helping to improve the quality of coverage and to strengthen benefits.

Current EVV tools also convey valuable information for identifying potential fraud, waste, and abuse—and to avoid it in the first place by making scheduling, plan of care compliance, and visit verification easier and more intuitive for caregivers. Provider agencies and payers have access to reporting and analytics empowering them to quickly spot and act on trends that may be indicative of FWA activity, which can be addressed either through caregiver training or working with the experienced vendor to refine software functionality or business rules. Enhanced reporting can also be made available or mandatory to identify conflicts in delivery, an essential point of FWA identification and investigation.

CMS's role would be to maximize these capabilities through incentives or mandates to enhance stakeholder adoption and compliance. The requirement for states to implement EVV and aggregation of data has been a positive factor by creating a nationwide foundation, but the relative lack of substantive consequences/penalties for low adoption has limited its success because of wide variances in compliance. Perhaps a first step would be to set the bar by establishing a national EVV compliance percentage, which could go a long way in getting the entire ecosystem on a level playing field, so the in-place framework can be used beyond just compliance.

With proven EVV tools established in place already, further measures by CMS to hold states and providers accountable for using them fully would enable the entire ecosystem to reap the benefits of more effective caregiving practices. The advantages would comprise not just better health and quality of life outcomes, but also cost savings from (1) earlier risk identification, leading to fewer emergency-room visits and prolonged hospital stays, and (2) more efficient utilization resulting from improved FWA oversight.

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Patient Privacy

As established SaaS platforms, the leading EVV systems have proven HIPAA compliance and numerous other security certifications based on their use of role-based access, encryption and network safeguards, and more. As such, these systems can be trusted to protect sensitive information about patients. Extending and incentivizing their use will reduce the risk of breaches or other unapproved access compared to introducing new, unfamiliar processes that potentially create additional points of vulnerability.

Convenience and Access

Maximizing the use of existing platforms that users have already learned also provides greater convenience and more reliable access. For most people, including a task or two in an existing, familiar workflow is easier than having to juggle yet another application.

Improve Outcomes

As noted above, and as desired by CMS, the security, convenience, and demonstrated reliability of these online platforms position them ideally to deliver capabilities that improve patients' health outcomes (the ultimate goal of any digital health product or service). In the case of EVV solutions already used by caregivers to clock in and out of home visits, more extensive/consistent adoption driven by stronger CMS requirements can lead to better real-time reporting of in-person observations of patients' health status, based on condition-specific prompts in the mobile application. The direct result will be faster, more frequent recognition of early warning signs and more proactive response options, reducing the need for hospital readmissions or other high-cost interventions.